

Antibiotic in Pregnancy and Lactation

Types of Antibiotics/ Antiviral/ Antiviral/Anti TB	FDA Pregnancy category	Lactation Considerations
Abacavir	C	Present in human milk; Not recommended
Acyclovir	B	Present in human milk; Compatible
Adefovir	C	Insufficient data; Not recommended
Amikacin	D	Present in human milk; Compatible
Amoxicillin	B	Present in human milk; Compatible
*Amoxicillin / Clavulanate	*B	Present in human milk; Compatible
Amphotericin B	B	Insufficient data; Not recommended
Ampicillin	B	Present in human milk; Compatible
Ampicillin / Sulbactam	B	Present in human milk; Compatible
Artesunate	Limited data for 1 st trimester	Insufficient data; Caution
Azithromycin	B	Present in human milk; Caution
Bacampicillin	B	Insufficient data
Benzathine Penicillin	B	Present in human milk; Caution
Benzylpenicillin	B	Present in human milk; Caution
Caspofungin	C	Insufficient data; Caution
Cefaclor	B	Insufficient data; Caution
Cefepime	B	Present in human milk; Caution
Cefoperazone	B	Insufficient data
Cefoperazone / Sulbactam	B	Insufficient data
Cefotaxime	B	Present in human milk; Caution
Ceftazidime	B	Present in human milk; Caution
Ceftriaxone	B	Present in human milk; Caution
Cefuroxime Axetil	B	Present in human milk; Caution
Cefuroxime Sodium	B	Present in human milk; Caution
Cephalexin Monohydrate	B	Present in human milk; Caution
Chloramphenicol	C	Present in human milk; Not recommended
Ciprofloxacin	C	Present in human milk; Not recommended
Clarithromycin	C	Present in human milk; Caution
Clindamycin	B	Present in human milk; Not recommended
Clofazimine	C	Present in human milk; Not recommended
Clotrimazole	B	Insufficient data
Cloxacillin	B	Present in human milk; Caution
Cycloserine	C	Present in human milk; Not recommended
Dapsone	C	Present in human milk; Not recommended
Didanosine	B	Insufficient data; Not recommended
Doxycycline	D	Present in human milk ; Insufficient data; Compatible for short courses (eg 10 days); Long- term therapy is not recommended
Efavirenz	D	Present in human milk; Not recommended
Ertapenem	B	Present in human milk; Caution

***Note: Amoxicillin/Clavulanate use during pregnancy is associated with neonatal necrotizing enterocolitis.⁴**

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Erythromycin	B	Present in human milk; Compatible
Ethambutol	C	Present in human milk; Compatible
Fluconazole	D	Present in human milk; Compatible
Flucytosine	C	Insufficient data
Fusidate sodium	C	Insufficient data
Ganciclovir	C	Insufficient data; Not recommended
Gentamicin	D	Present in human milk; Compatible
Griseofulvin	C	Insufficient data; Not recommended
Imipenem / Cilastatin	C	Present in human milk; Caution
Indinavir	C	Insufficient data; Not recommended
Isoniazid	C	Present in human milk; Compatible
Itraconazole	C	Present in human milk; Caution
Kanamycin	D	No data available
Ketoconazole	C	Present in human milk; Caution
Lamivudine	C	Insufficient data; Not recommended
Levofloxacin	C	Present in human milk; Not recommended
Linezolid	C	Present in human milk; Caution
Lopinavir / Ritonavir	C	Present in human milk; Not recommended
Meropenem	B	Present in human milk; Caution
Metronidazole	B	Present in human milk; Not recommended
Miconazole	C	Insufficient data; Caution
Minocycline	D	Present in human milk ; Insufficient data; Compatible for short courses (eg 10 days); Long- term therapy is not recommended
Netilmicin	D	Insufficient data
Nevirapine	C	Present in human milk; Not recommended
Nitrofurantoin	B	Present in human milk; Not recommended
Nystatin	C	Insufficient data; Caution
Ofloxacin	C	Present in human milk; Not recommended
Phenoxymethyl penicillin	B	Present in human milk; Compatible
Piperacillin	B	Present in human milk; Compatible
Piperacillin / Tazobactam	B	Present in human milk; Compatible
Benzympenicillin	B	Present in human milk; Caution
Pyrazinamide	C	Present in human milk; Caution
Ribavirin	X	Insufficient data
Rifampicin	C	Compatible; may cause diarrhea in infant. Monitor infant for jaundice
Ritonavir	B	Insufficient data; Not recommended
Stavudine	C	Insufficient data; Not recommended
Streptomycin	D	Present in human milk; Not recommended
Sulphamethoxazole / Trimethoprim	D	Present in human milk; Caution; Not recommended in infants <2 months
Terbinafine HCL	B	Present in human milk; Not recommended
Tetracycline	D	Present in human milk ; Insufficient data; Compatible for short courses (eg 10 days); Long- term therapy is not recommended
Tinidazole	C	Present in human milk; Not recommended
Trimethoprim	C	Present in human milk; Caution

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Vancomycin	B (oral)	Present in human milk; Caution
Voriconazole	D	Insufficient data; Not recommended
Zidovudine	C	Present in human milk; Not recommended

References:

1. Anderson PO, Saubaran JB. Modeling drug passage into human milk. *Clin Pharmacol Ther.* 2016;100:42-52. Retrieved 24 Jun 2019 from <https://uptodate.com/breast-feeding> considerations
2. Chung AM, Reed MD, Blumer JL. "Antibiotics and Breast-Feeding: A Critical Review of the Literature". *Paediatr Drugs.* 2002;4:817-837. Retrieved 24 Jun 2019 from <https://uptodate.com/breast-feeding> considerations
3. Drugs.com Pregnancy / Breastfeeding
4. Kenyon SL, Taylor DJ, Tarnow-Mordi W. Broad-spectrum antibiotics for preterm, prelabour rupture of fetal membranes: the ORACLE I randomised trial. ORACLE Collaborative Group. *Lancet* 2001;357(9261):979-988.